SUMMARY OF PRODUCT CHARACTERISTICS

1. NAME OF THE MEDICINE

AZmycin Tablet 500 mg

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each tablet contains 500 mg of azithromycin (as dihydrate).

For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Film-coated tablet

White, elongated, biconvex tablets, scored on one side and with the inscription 500 on the other side.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Azithromycin is indicated in the treatment of the following infections caused by susceptible microorganisms (see Pharmacodynamic properties):

- Infections of the lower respiratory tract: bronchitis and mild to moderate severe community acquired pneumonia.
- Infections of the upper respiratory tract: sinusitis and pharyngitis/tonsillitis.
- Acute otitis media.
- Infections of the skin and soft tissues
- Sexually transmitted diseases: uncomplicated urethritis and cervicitis produced by *Chlamydia trachomatis*, chancroid

Official recommendation regarding the appropriate use and prescription of antimicrobial agents should be taken into consideration.

Azithromycin is not the empirical treatment of choice for these infections in areas where the prevalence of resistant isolates is 10% or more (see Pharmacodynamic properties).

4.2 Posology and method of administration

Oral azithromycin should be administered as a single daily dose. The period of dosing with regard to infection is given below. Azithromycin tablets can be taken with or without food.

Adults

For the treatment of sexually transmitted diseases caused by Chlamydia trachomatis, Haemophilus ducreyi, or susceptible Neisseria gonorrhoea, the dose is 1000 mg as a single oral dose.

For prophylaxis against MAC infections in patients infected with the human immunodeficiency virus (HIV), the dose is 1200 mg once per week.

For the treatment of DMAC infections in patients with advanced HIV infection, the recommended dose is 600 mg once a day. Azithromycin should be administered in combination with other antimycobacterial agents that have shown in vitro activity against MAC, such as ethambutol at the approved dose.

For the treatment of adult patients with CAP due to the indicated organisms, the recommended dose of intravenous azithromycin is 500 mg as a single daily dose by the IV route for at least two days. Intravenous therapy should be followed by oral azithromycin as a single daily dose of 500 mg to complete a 7 to 10 day course of therapy. The timing of the conversion to oral therapy should be done at the discretion of the physician and in accordance with clinical response.

For the treatment of adult patients with PID due to the indicated organisms, the recommended dose of intravenous azithromycin is 500 mg as a single dose by the IV route for one or two days. Intravenous therapy should be followed by azithromycin by the oral route at a single daily dose of 250 mg to complete a 7day course of therapy. The timing of the conversion to oral therapy should be done at the discretion of the physician and in accordance with clinical response. If anaerobic microorganisms are suspected of contributing to the infection, an antimicrobial anaerobic agent may be administered in combination with azithromycin.

For all other indications in which the oral formulation is administered, the total dosage of 1500 mg should be given as 500 mg daily for 3 days. As an alternative, the same total dose can be given over 5 days with 500 mg given on day 1, then 250 mg daily on days 2 to 5.

Children

The maximum recommended total dose for any treatment is 1500 mg for children. In general, the total dose in children is 30 mg/kg. Treatment for pediatric streptococcal pharyngitis should be dosed at a different regimen (see below).

The total dose of 30 mg/kg should be given as a single daily dose of 10 mg/kg daily for 3 days, or given over 5 days with a single daily dose of 10 mg/kg on day 1, then 5 mg/kg on days 2-5. As an alternative to the above dosing, treatment for children with acute otitis media can be given as a single dose of 30 mg/kg.

For pediatric streptococcal pharyngitis, azithromycin given as a single dose of 10 mg/kg or 20 mg/kg for 3 days has been shown to be effective. However, a daily dose of 500 mg must not be exceeded. Penicillin is the usual drug of choice in the treatment of Streptococcus pyogenes pharyngitis, including prophylaxis of rheumatic fever.

Azithromycin tablets should only be administered to children weighing more than 45 kg. Safety and efficacy for the prevention or treatment of MAC in children have not been established. Based on pediatric pharmacokinetic data, a dose of 20 mg/kg would be similar to the adult dose of 1200 mg but with a higher Cmax.

Elderly

The same dosage as in adult patients is used in the elderly.

Patients with Renal Impairment

No dose adjustment is necessary in patients with mild to moderate renal impairment (GFR 10-80 ml/min). Caution should be exercised when azithromycin is administered to patients with severe renal impairment (GFR < 10 ml/min).

Patients with Hepatic Impairment

The same dosage as in patients with normal hepatic function may be used in patients with mild to moderate hepatic impairment.

4.3 Contraindications

Hypersensitivity to azithromycin, macrolide or ketolide antibiotics or to any of the excipients.

4.4 Special warnings and precautions for use

Allergic reactions
Severe allergic reactions have been reported in rare cases, such as angioedema and anaphylaxis. Some of these reactions with azithromycin have resulted in recurrent symptoms which have required longer period of observation and treatment.

In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCARs) [e.g. Stevens-Johnson Syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) and acute generalised exanthematous pustulosis (AGEP)], AZmycin should be discontinued immediately and appropriate treatment should be urgently initiated.

Use in infants
Infantile hypertrophic pyloric stenosis (IHPS) has been reported following the use of azithromycin in infants (treatment up to 42 days of life). Parents and caregivers should be informed to contact their physician if vomiting and/ or irritability with feeding occurs.

Use in renal impairment
In patients with with mild to moderate renal impairment (glomerular filtration rate 10-80 ml/min), no dose adjustment is required. Precaution is recommended in patients with severe renal impairment (glomerular filtration rate < 10 ml/min) as systemic exposure may be increased (see section Pharmacokinetics properties).

Use in hepatic impairment
There is no information available about the use of azithromycin in patients with severe liver disease (Child-Pugh class C) (see section 5.2). Treatment with azithromycin should be interrupted if severe liver failure occurs. If is administration is believed to be essential, the evolution of liver function tests should be periodically monitored.

Ergotamine derivatives and azithromycin

It has been reported that the concomitant use of ergot alkaloids and macrolide antibiotics accelerated the development of ergotism. Although no interaction studies have been conducted with ergot alkaloid derivatives and azithromycin, the development of ergotism is possible, so their combined administration is not recommended.

Prolongation of the QT interval
Prolonged cardiac repolarization and QT interval, imparting a risk of developing cardiac arrhythmia and torsades de pointes, have been seen in treatment with macrolides, including azithromycin. Prescribers should consider the risk of QT prolongation, which can be fatal, when weighing the risks and benefits of azithromycin for at-risk groups including:

- Patients with congenital or documented QT prolongation
- Patients currently receiving treatment with other active substances known to prolong QT interval, such as antiarrhythmics of Classes IA and III, antipsychotic agents, antidepressants, and fluoroquinolones
- Patients with electrolyte disturbance, particularly in cases of hypokalemia and hypomagnesemia
- Patients with clinically relevant bradycardia, cardiac arrhythmia or cardiac insufficiency
- Elderly patients: elderly patients may be more susceptible to drug-associated effects on the QT interval

The following considerations should be taken into account before prescribing azithromycin:

Azithromycin tablets are not appropriate for the treatment of severe infections in which appropriate plasma concentrations have to be rapidly reached.

In areas with a high incidence of erythromycin resistance, it is important to consider the evolution of sensitivity to azithromycin and other antibiotics.

As for other macrolide antibiotics, a high prevalence rate (>30%) of *Streptococcus pneumoniae* isolates resistant to azithromycin has been described in some European Union countries. This should be considered when treating infections caused by this microorganism.

Pharyngitis/tonsillitis
Azithromycin is not the first choice treatment for pharyngitis and tonsillitis caused by *Streptococcus pyogenes*. Penicillin is the treatment of choice for these infections and for the prophylaxis of acute rheumatic fever.

Acute otitis media
Azithromycin is often not the first choice of treatment for acute otitis media.

Sinusitis
Azithromycin is often not the first choice of treatment for sinusitis. There is limited information available about the treatment of sinusitis in children and adolescents under 16 years of age.

Infected burns
Azithromycin is not indicated for the treatment of infected burn wounds.

Sexually transmitted diseases
A concomitant infection caused by *T. pallidum* should be excluded in cases of sexually transmitted diseases.

Superinfections

Attention should be paid to possible symptoms of overinfection caused by non-susceptible microorganisms such as fungi. The development of superinfection may require the interruption of treatment with azithromycin and the use of appropriate measures.

Neurological or mental diseases
Azithromycin should be administered with caution to patients with neurological or mental diseases.

Pseudomembranous colitis
Cases of pseudomembranous colitis have been described after the use of macrolide antibiotics. This diagnosis should be considered in patients who get diarrhoea after starting the treatment with azithromycin. The use of anti-peristaltic agents is contraindicated in azithromycin-induced pseudomembranous colitis.

Long term treatment
There is no experience regarding the safety and efficacy of long-term treatment with azithromycin for the specified indications. Treatment with another antibiotic should be considered in case of frequent recurring infections.

AZmycin 500 mg film-coated tablets do not contain gluten.

4.5 Interactions with other medicaments

Antacids
No effect on overall bioavailability was seen although peak serum concentrations were reduced by approximately 25%. In patients receiving both oral azithromycin and antacids, the drugs should not be taken simultaneously.

Cetirizine
Coadministration of a 5-day regimen of azithromycin with cetirizine 20 mg at steady-state resulted in no pharmacokinetic interaction and no significant changes in the QT interval.

Didanosine (Dideoxyinosine)
Coadministration of 1200 mg/day azithromycin with 400 mg/day didanosine in HIV-positive subjects did not appear to affect the steady-state pharmacokinetics of didanosine as compared with placebo.

Digoxin
Some of the macrolide antibiotics have been reported to impair the microbial metabolism of digoxin in the gut in some patients. In patients receiving concomitant azithromycin, a related azalide antibiotic, and digoxin the possibility of raised digoxin levels should be borne in mind.

Zidovudine
Single 1000 mg doses and multiple 1200 mg or 600 mg doses of azithromycin had little effect on the plasma pharmacokinetics or urinary excretion of zidovudine or its glucuronide metabolite. However, administration of azithromycin increased the concentrations of phosphorylated zidovudine, the clinically active metabolite, in peripheral blood mononuclear cells. The clinical significance of this finding is unclear, but it may be of benefit to patients.

Azithromycin does not interact significantly with the hepatic cytochrome P450 system. It is not believed to undergo the pharmacokinetic drug interactions as seen with erythromycin and other macrolides. Hepatic cytochrome P450 induction or inactivation via cytochrome-metabolite complex does not occur with azithromycin.

Ergot
Due to the theoretical possibility of ergotism, the concurrent use of azithromycin with ergot derivatives is not recommended.

Pharmacokinetic studies have been conducted between azithromycin and the following drugs known to undergo significant cytochrome P450 mediated metabolism.

Atorvastatin
Coadministration of atorvastatin (10 mg daily) and azithromycin (500 mg daily) did not alter the plasma concentrations of atorvastatin.

Carbamazepine
No significant effect was observed on the plasma levels of carbamazepine or its active metabolite in patients receiving concomitant azithromycin.

Cimetidine
In a pharmacokinetic study investigating the effects of a single dose of cimetidine, given 2 hours before azithromycin, on the pharmacokinetics of azithromycin, no alteration of azithromycin pharmacokinetics was seen.

Coumarin-Type Oral Anticoagulants
There have been reports of potentiated anticoagulation subsequent to coadministration of azithromycin and coumarin-type oral anticoagulants. Although a causal relationship has not been established, consideration should be given to the frequency of monitoring prothrombin time when azithromycin is used in patients receiving coumarin-type oral anticoagulants.

Cyclosporin
Caution should be exercised before considering concurrent administration of cyclosporin and azithromycin. If coadministration of these drugs is necessary, cyclosporin levels should be monitored and the dose adjusted accordingly.

Efavirenz
Coadministration of a 600 mg single dose of azithromycin and 400 mg efavirenz daily for 7 days did not result in any clinically significant pharmacokinetic interactions.

Fluconazole
Coadministration of a single dose of 1200 mg azithromycin did not alter the pharmacokinetics of a single dose of 800 mg fluconazole. Total exposure and half-life of azithromycin were unchanged by the coadministration of fluconazole, however a clinically insignificant decrease in Cmax (18%) of azithromycin was observed.

Indinavir
Coadministration of a single dose of 1200 mg azithromycin had no statistically significant effect on the pharmacokinetics of indinavir administered as 800 mg three times daily for 5 days.

Methylprednisolone

Azithromycin had no significant effect on the pharmacokinetics of methylprednisolone.

Midazolam
Coadministration of azithromycin 500 mg/day for 3 days did not cause clinically significant changes in the pharmacokinetics and pharmacodynamics of a single 15 mg dose of midazolam

Nelfinavir
Coadministration of azithromycin (1200 mg) and nelfinavir at steady state (750 mg three times daily) resulted in increased azithromycin concentrations. No clinically significant adverse effects were observed and no dose adjustment is required.

Rifabutin
Coadministration of azithromycin and rifabutin did not affect the serum concentrations of either drug.

Neutropenia was observed in subjects receiving concomitant treatment of azithromycin and rifabutin. Although neutropenia has been associated with the use of rifabutin, a causal relationship to combination with azithromycin has not been established.

Sildenafil
There was no evidence of an effect of azithromycin (500mg daily for 3 days) on the AUC and Cmax of sildenafil or its major circulating metabolite.

Terfenadine
Pharmacokinetic studies have reported no evidence of an interaction between azithromycin and terfenadine. There have been rare cases reported where the possibility of such an interaction could not be entirely excluded; however, there was no specific evidence that such an interaction had occurred.

Theophylline
There is no evidence of a clinically significant pharmacokinetic interaction when azithromycin and theophylline are co-administered to healthy volunteers.

Triazolam
Coadministration of azithromycin with 0.125 mg triazolam had no significant effect on any of the pharmacokinetic variables for triazolam compared to triazolam and placebo.

Trimethoprim/sulfamethoxazole
Coadministration of trimethoprim/sulfamethoxazole DS (160 mg/800 mg) for 7 days with azithromycin 1200 mg on Day 7 had no significant effect on peak concentrations, total exposure or urinary excretion of either trimethoprim or sulfamethoxazole. Azithromycin serum concentrations were similar to those seen in other studies.

4.6 Pregnancy and lactation

Pregnancy
The safety of azithromycin has not been confirmed in relation to its use during pregnancy. Therefore, azithromycin should only be used during pregnancy in situations which are not life-threatening.

Lactation

Azithromycin is excreted in mother's milk, and it is not known whether it can cause undesirable reactions in the infant. It is therefore advisable to suspend breastfeeding during treatment with azithromycin. The possible consequences for the infant, among others, could be diarrhoea, fungal infections of the mucous membrane and sensitisation. It is advisable to discard mother's milk during treatment and up to 2 days after the end of treatment. Breastfeeding can subsequently be resumed.

4.7 Undesirable effects

Azithromycin is well tolerated with a low incidence of side effects.

Blood and Lymphatic System Disorders
Transient episodes of mild neutropenia have occasionally been observed in clinical trials, although a causal relationship to azithromycin has not been established.

Ear and Labyrinth Disorders
Hearing impairment (including hearing loss, deafness and/or tinnitus) has been reported in some patients receiving azithromycin. Many of these have been associated with prolonged use of high doses in investigational studies. In those cases where follow-up information was available the majority of these events were reversible.

Gastrointestinal Disorders
Nausea, vomiting, diarrhea, loose stools, abdominal discomfort (pain/cramps), and flatulence.

Hepatobiliary Disorders
Abnormal liver function.

Skin and Subcutaneous Tissue Disorders
Allergic reactions including rash and angioedema.

Frequency not known: severe cutaneous adverse reactions (SCARs) including Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP).

General Disorders and Administration Site Conditions
Local pain and inflammation at the site of infusion.

The following undesirable effects have been reported in association with DMAC prophylaxis and treatment:

The most frequent adverse reactions in HIV-infected patients receiving azithromycin for prophylaxis for DMAC were diarrhea, abdominal pain, nausea, loose stools, flatulence, vomiting, dyspepsia, rash, pruritus, headache, and arthralgia.

When azithromycin 600 mg is given daily for the treatment of DMAC infection for prolonged periods, the most frequently reported treatment-related side effects are abdominal pain, nausea, vomiting, diarrhea, flatulence, headache, abnormal vision, and hearing impairment.

In post-marketing experience, the following additional undesirable effects have been reported:

Infections and Infestations

Moniliasis and vaginitis.

Blood and Lymphatic System Disorders
Thrombocytopenia.

Immune System Disorders
Anaphylaxis (rarely fatal).

Metabolism and Nutrition Disorders
Anorexia.

Psychiatric Disorders
Aggressive reaction, nervousness, agitation, and anxiety.

Nervous System Disorders
Dizziness, convulsions (as seen with other macrolides), headache, hyperactivity, hypoesthesia, paresthesia, somnolence, and syncope. There have been rare reports of taste/smell perversion and/or loss. However, a causal relationship has not been established.

Ear and Labyrinth Disorders
Vertigo.

Cardiac Disorders
Palpitations and arrhythmias including ventricular tachycardia have been reported. There have been rare reports of QT prolongation and torsades de pointes (see section Special warnings and precautions for use).

Vascular Disorders
Hypotension

Gastrointestinal Disorders
Vomiting/diarrhea (rarely resulting in dehydration), dyspepsia, constipation, pseudomembranous colitis, pancreatitis, rare reports of tongue discoloration and infantile hypertrophic pyloric stenosis.

Hepatobiliary Disorders
Hepatitis and cholestatic jaundice have been reported, as well as rare cases of hepatic necrosis and hepatic failure, which have rarely resulted in death. However, a causal relationship has not been established.

Skin and Subcutaneous Tissue Disorders
Allergic reactions including pruritus, rash, photosensitivity, edema, urticaria, and angioedema. Rarely, serious skin reactions including erythema multiforme, Stevens-Johnson syndrome, and toxic epidermal necrolysis have been reported.

Musculoskeletal and Connective Tissue Disorders
Arthralgia.

Renal and Urinary Disorders
Interstitial nephritis and acute renal failure.

General Disorders and Administration Site Conditions

Asthenia has been reported, although a causal relationship has not been established; fatigue, and malaise.

4.8 Overdose

The undesirable effects occurring with higher than the recommended doses were the same as those known with normal doses. The characteristic symptoms of an overdose with macrolide antibiotics include reversible hearing loss, intense nausea, vomiting and diarrhoea. In case of overdose, the administration of activated charcoal, general symptomatic measures and general vital function support measures are indicated when necessary.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Antibiotics for systemic use, macrolides. ATC code: J01 FA10

Azithromycin is the first of a subclass of macrolides antibiotics, known as azalides, and is chemically different from erythromycin. Chemically it is derived by insertion of a nitrogen atom into the lactone ring of erythromycin A. The mode of action of azithromycin is inhibition of protein synthesis in bacteria by binding to the 50s ribosomal subunit and preventing translocation of peptides.

Azithromycin demonstrates activity in vitro against a wide range of bacteria including:

Gram-positive Aerobic Bacteria
Staphylococcus aureus, *Streptococcus pyogenes* (group A beta-hemolytic streptococci), *Streptococcus pneumoniae, alpha-hemolytic streptococci* (viridans group) and other streptococci, and *Corynebacterium diphtheriae*. Azithromycin demonstrates cross-resistance with erythromycin-resistant Gram-positive strains, including *Streptococcus faecalis* (enterococcus) and most strains of methicillinresistant staphylococci.

Gram-negative Aerobic Bacteria
Haemophilus influenzae, Haemophilus parainfluenzae, Moraxella catarrhalis, Acinetobacter species, Yersinia species, Legionella pneumophila, Bordetella pertussis, Bordetella parapertussis, Shigella species, Pasteurella species, Vibrio cholerae and parahaemolyticus, Plesiomonas shigelloides. Activities against *Escherichia coli, Salmonella enteritidis, Salmonella typhi, Enterobacter* species, *Aeromonas hydrophila* and *Klebsiella* species are variable and susceptibility tests should be performed. *Proteus* species, *Serratia* species, *Morganella* species, and *Pseudomonas aeruginosa* are usually resistant.

Anaerobic Bacteria
Bacteroides fragilis and *Bacteroides* species, *Clostridium perfringens*, *Peptococcus* species and *Peptostreptococcus* species, *Fusobacterium necrophorum* and *Propionibacterium acnes*.

Organisms of Sexually Transmitted Diseases
Azithromycin is active against *Chlamydia trachomatis* and also shows good activity against *Treponema pallidum*, *Neisseria gonorrhoeae*, and *Haemophilus ducreyi*.

Other Organisms
Borrelia burgdorferi (Lyme disease agent), *Chlamydia pneumoniae, Mycoplasma pneumoniae, Mycoplasma hominis, Ureaplasma urealyticum, Campylobacter* species and *Listeria monocytogenes*.

Opportunistic Pathogens Associated with HIV Infections
Mycobacterium avium-intracellulare complex, Pneumocystis carinii and *Toxoplasma gondii.*

Commonly susceptible species :

Aerobic Gram-positive bacteria:
Staphylococcus aureus, Streptococcus agalactiae, Streptococci (Groups C, F, G) and Viridans group streptococci.

Aerobic Gram-negative bacteria:
Bordetella pertussis, Haemophilus ducreyi, Haemophilus influenzae, Haemophilus parainfluenzae, Legionella pneumophila, Moraxella catarrhalis and *Neisseria gonorrhoeae*.

Other
Chlamydia pneumoniae, Chlamydia trachomatis, Mycoplasma pneumonia and *Ureaplasma urealyticum.*

Species for which acquired resistance:
Aerobic Gram-positive bacteria: Streptococcus pneumoniae, Streptococcus pyogenes

Note: Azithromycin demonstrates cross-resistance with erythromycin-resistant gram-positive strains.

Inherently resistant organisms: Enterobacteriaceae, *Pseudomonas*

5.2 Pharmacokinetic properties

Absorption
Azithromycin is well absorbed following oral administration and rapidly passes from serum to tissues and various organs. After a single 500 mg oral dose of azithromycin, 37% of the drug is absorbed, and a peak plasma concentration (0.41µg/ml) is achieved in 2-3 hours.

Distribution
Azithromycin is widely distributed throughout the body, achieving high tissue concentration (up to 50 times higher than observed concentration in plasma). Volume of distribution is approximately 31 l/kg. Azithromycin is rapidly distributed to a wide range of tissues and achieves high tissue concentrations ranging between 1 and 9µg/ml, depending on the tissue. Therapeutic concentrations of azithromycin are maintained in tissues for five to seven days after the ingestion of last oral dose. Azithromycin achieves very high concentrations in phagocytic cells, which migrate to infected sites.

Elimination
Plasma terminal elimination half-life closely reflects the tissue depletion half-life of 2-4 days. Biliary excretion of azithromycin is a major route of elimination. Approximately 50% of biliary excretion is in the form of unchanged compound. The other half are 10 metabolites formed by N- and O-demethylation, by hydroxylation of desosamine and aglycone rings, and by cleavage of the cladinose conjugate. Comparison of HPLC and microbiological assay suggests that metabolites play no part in the microbiological activity of azithromycin. Approximately 6% of administered dose is excreted in urine.

<u>Pharmacokinetics in Special Patient Groups</u>

Elderly
In elderly volunteers (> 65 years), slightly higher (30%) AUC values were seen than in younger volunteers (< 45 years), but this is not considered clinically significant and hence no dose adjustment is recommended.

Hepatic Impairment
In patients with mild to moderate hepatic impairment, there is no evidence of a marked change in serum pharmacokinetics of azithromycin compared to those with normal hepatic function. In these patients urinary clearance of azithromycin appears to increase, perhaps to compensate for reduced hepatic clearance.

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Anhydrous dibasic calcium hydrogen phosphate
Hypromellose
Pregelatinised starch
Croscarmellose sodium
Magnesium stearate (E470)
Sodium lauryl sulfate
Titanium dioxide (E 171)
Polydextrose
Macrogol 4000

6.2 Shelf life

5 years.

6.3 Special precautions for storage

Store below 30°C and protect from direct sunlight.

6.4 Nature and contents of container

The tablets are packed in PVC/Aluminium blisters.

This medicinal product is presented in packs with 3 tablets.

7. MANUFACTURER

Neuraxpharm Pharmaceuticals, S. L.
Avda. Barcelona, 69
08970 Sant Joan Despi (Barcelona)
Spain

8. PRODUCT REGISTRATION HOLDER

MD Healthcare Sdn Bhd

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65 Jalan Trus, 80000 Johor Bahru, Malaysia

9. DATE OF REVISION OF THE TEXT

27 October 2023